جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ام الهنا عبد المعطي بيومي عبدالمحسن
المحافظة : القاهره **الحي / المدينة :** حلوان **العنوان :** ش شعراوى
رقم البطاقة : 27404010105141 **رقم التليفون :** 01126209098 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 01-04-1974 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ام الهنا عبد المعطي بيومي عبدالمحسن
National ID: 27404010105141

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

[[DOCTYPE:IHC]]
Immunohistochemistry

Patient Name	: الأستاذه / ام الهنا عبدالمعطى بيومى عبدالمحسن	Request No.	: 13007700
Patient No.	: 27404010105141	Request Date	: 16/04/2022
Path No.	: 1018/2022	Verification Date	: 26/04/2022
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 48 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical data:
Left breast lesion

Nature of Specimen :
Tru cut biopsy

Gross:
Multiple grayish white tissue,cores, longest measured up to 1 cm long. Totally submitted.
Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic:
histopathological examination of sections prepared from the specimen received reveals cores of an infiltrating malignant epithelial growth. The tumour is made up of groups and small sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate [grade II].
Biomarkers:
ER: Strong nuclear reaction in 70% of tumor cells (score 8/8)
PR: Strong nuclear reaction in 70% of tumor cells (score 8/8)
Her-2: Weak incomplete membrane reaction of tumor cells (score 1+)
Ki 67: 10 % nuclear reaction

agnosis:
breast, Tru cut biopsy;
asive duct carcinoma, GII.
Positive (8/8)

Verified by :
Dr.Marwa mahmoud

Room No. (G - First Floor Building No. 897, Nile Corniche - Mit El Qudsen- Cairo, Egypt

Name: أم الهنا عبد المعطي بيومي	
MR. : 5017	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 52-year-old individual with a diagnosis of luminal NACTH-positive breast carcinoma, staged as T2N1 following a left-sided mastectomy performed in January 2023. Post-operative management included placement of a port and initiation of Nolvadex (tamoxifen) for five years, followed by an aromatase inhibitor for two years. The current regimen consists of Nolvadex combined with supplemental therapy, and the patient has been on this treatment for three months. The plan is to continue the current hormonal therapy as outlined.

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : اسماء محمد مصطفي نصر

يعتمد ,

Patient Name	: الأستاذه / ام الهنا عبدالمعطى بيومى عبدالمحسن	Request No.	: 13003368
Patient No.	: 27404010105141	Request Date	: 11/01/2023
Path No.	: 89/2023	Verification Date	: 24/01/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 48 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Left breast cancer, post chemotherapy; Left simple mastectomy and SLNs biopsy specimens:

- Invasive tubular carcinoma, grade I
- Free nipple and areola
- Free surgical margins
- Positive metastatic deposits in 1/4 SLNs with capsular invasion
- Pathologic partial response
- Stage= ypT2 ypN1

Prof.Dr. Eman Gouda
Dr. Heba Mostafa

Verified by:
walaa ghoniem

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / ام الهنا عبدالمعطى بيومى عبدالمحسن	Request No.	: 13003368
Patient No.	: 27404010105141	Request Date	: 11/01/2023
Path No.	: 89/2023	Verification Date	: 24/01/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 48 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data:Left breast cancer, post chemotherapy.

Nature of the specimen: Left simple mastectomy and left SLNs biopsy specimens.

Gross Examination:
- Procedure: Left simple mastectomy specimen; measured 24x16x7 cm, covering skin measured 17x13 cm, nipple and areola were unremarkable grossly.
- Localization: LIQ
- Tumor extent
  - o Number: Ill defined rubbery graiysh white TBS with central firm area
  - o Size: 5x3x2 cm.
  - o Skin involvement: 1.5 cm from skin
- Distance from margins: 2 cm away from deep margin
- Lymph nodes number and size: Three fibrofatty tissue pieces ranging in size from 3x2 cm up to 4.5x2.5 cm, entangling 4 lymph nodes up to 1x1 cm.
- Additional pathologic findings:N/A
- Immunohistochemistry technique: N/A
- Frozen section: Four lymph nodes with therapy effect for paraffin examination.

Microscopic examination:
- Histological type:Invasive tubular carcinoma, cellularity 60%
- Grade (differentiation+ nuclear pleomorphism + mitosis): I (2+2+1)
- Lymph node involvement: Positive metastatic deposits in 1/4 SLNs with capsular invasion
- Resection Margins: Free
- Histologic response to therapy: Pathologic partial response
- Additional pathologic findings: Free nipple and areola

Diagnosis:

Verified by:
walaa ghoniem

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient name: Om El Hana Abd El Aty
Date: 10.4.2022

Full Field Digital bilateral breast mammography & Complementary Ultrasound

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)

Report:

- Heterogeneously dense breast parenchyma with nodulation and mottling impressive of adenosis (ACR C)
- No suspicious micro calcifications or architectural distortion detected.
- Bilateral scattered variable sized cysts some of them show echogenic content.
- Left breast 2 o'clock ill defined area of aletered parenchyma echogenicty with tracts and pockets extending towards the skin showing echogenic internal content extending to the nipple.
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Bilateral fibrocystic mammary changes.
- Left breast 2 o'clock pathological process likely inflammatory rather than being neoplastic for histopathological assessment (BIRADS 4a)

Best regards,
Dr. Aya Deabes,MSc.

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Om Alhana Abd Al Moaty Bayomy Abd Al Mohsen	Date : 24/12/2022
Exam : MAMMOGRAPHY BOTH	Age : 48 Year(s)
Medical ID: 1398/2022	Pat. ID: 14953

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Left BC, on neoadjuvent

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- The breasts are heterogeneously dense which may obscure small lesions (ACR C)
- Left 2:00 spiculated lesion measuring 2.2x2.2 cm seen 5.2 cm from the nipple, surrounded with multiple well-circumscribed lesions of high density. US showed 2:00 spiculated hypoechoic solid lesion measuring 1.34x1.67 cm, with multiple dilated ducts extending from the nipple toward the lesion, showing internal echogenic contents.
- No right suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- Complementary ultrasound showed no right solid mass lesions.
- Bilateral multiple scattered anechoic simple cysts, with few in the left breast showing internal echogenic contents
- Normal skin thickness and nipple-areola complex.
- Bilateral non-specific axillary LNs.

Opinion:

- Left UOQ subtle progress response of UOQ lesion (BIRADS6)
- Left Retroareolar dilated ducts with ?? inspissated secretions (BIRADS3)
- Bilateral fibrocystic changes (BIRADS 2)

Yours truly

Signature Dr. Yasmin Nada

24/12/2022 11:03 AM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Name: om el hana abd el moaty. Date: 14.06.2022

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY : left breast IDC grade II

TECHNIQUE: CC & MLO low energy & recombined (post contrast) images

FINDINGS :

Background Uptake: Mild

LEFT BREAST:

- **Pre-contrast images**: UOQ area of asymmetry.
- **Post- contrast images**:
- an area of non-mass enhancement is seen at the UOQ, measures 11 x 8 x 3 cm in diameter.
- Normal skin / nipple / areolar complex.
- Pathological axillary lymph nodes (at least 3) level I.

RIGHT BREAST:

- Dense fibroglandular pattern (ACR C).
- No masses, asymmetry or distortion or suspicious microcalcifications
- Benign calcific foci are seen.
- Post contrast images show, no pathological uptake.
- Normal skin / nipple / areolar complex.

IMPRESSION :

LEFT BREAST: (BIRADS 6= Pathology proved malignant) UOQ malignant non mass enhancement.

RIGHT BREAST: (BIRADS 1= Negative) normal study

Dr Hanaa Abou El Magd

ت / فاكس: ٠٢٢٣٦٤٥٨١٣
Tel, Fax: 0223645813

٣٠ ، ١ ش كورنيش النيل ، قصر الخليج ، القاهرة
1053 Corniche El Nil St. Fom Al Khaleeg, Cairo

Mammogram
[[DOCTYPE:MAMMOGRAM]]

[redacted]

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. **Chest:**
   - Diffuse pathological nodular enhancement of the left breast. Associated few prominent left axillary LNs are seen reaching 1.3 cm.
   - Few bilateral tiny subpleural pulmonary nodules are seen (2-3 mm).
   - Bilateral LL, posterior subpleural fine fibrotic bands.
   - Otherwise, No sizable pulmonary masses, areas of consolidation or cavitary lesions.
   - Few prominent superior mediastinal (pretracheal and retrosternal) lymph nodes are seen, largest measures 1 cm likely reactive .
   - No evidence of pleural effusion, pleural thickening or masses seen.
   - Normal cardiac size. No pericardial effusion detected.
   - Normal CT appearance of the thoracic aorta and the great vessels.
   - Right thyroid lobe, hypodense nodule is seen measuring 1x 0.7 cm.
2. **Abdomen and pelvis:**
   - The liver is mildly enlarged with homogenous CT texture. No focal lesions or contour irregularities.
   - Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
   - No intra-hepatic or extra-hepatic biliary radicles dilatation.
   - Bulky spleen with no focal lesions seen.
   - Normal size, parenchymal thickness & density of both kidneys with no back-pressure changes. Right renal upper calyceal tiny hyperdense gravel.
   - Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
   - Under filling of the urinary bladder with no stones.
   - Normal appearance of the other pelvic organs. IUD is seen in place.
   - No abdominal or pelvic lymphadenopathy or collections.
   - No suspicious lytic or sclerotic bone lesions.

OPINION:

- Diffuse pathological nodular enhancement of the left breast with associated few prominent left axillary lymph nodes, for sono-mammographic correlation.
- Few bilateral tiny subpleural pulmonary nodules ; likely insignificant. Otherwise, no definite pulmonary metastatic deposits
- Mild hepatomegaly. No focal lesions.
- No suspicious lytic or sclerotic bone lesions.

Dr. Peter Fahmy, MSc

Dr.m.gomma.mg

CT Scan
[[DOCTYPE:CT_SCAN]]

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

البنت المصرية هي صحة مصر

MDT FORM 11 / 2 /2023 GB

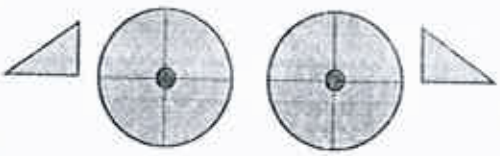
Name	ام الضاوي بلعطي يون	MRN	١٣٩٣
		ID	٤٠٢٢
Age	٤٩	Phone	٠١١٤٠٤٠١٠٥١٤٧
Menopausal status		Marital status	٠١١٥٦-٤٩٢٤٤
NRS SCORE	2	Family history	٠١١٢٧٥٨٠٥٦٧
BMI	25	REASON OF MDT	
CO morbidity			

Clinical Data

BX:

ER:	PR:	HER 2:	Ki67:
8/8	8/8	+1	10%

R L



Sonomammography:

Mets Workup:

TNM

Recommendations:

11/2/2023 → • for PORT
• 5 ys Tam then A.I 5 y. (excluded)
• If axilla U/S (to confirm no residual) before RTH

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Om Alhana Abd Al Moaty Bayomy Abd Al Mohsen	Date : 24/12/2022
Exam : MAMMOGRAPHY BOTH	Age : 48 Year(s)
Medical ID: 1398/2022	Pat. ID: 14953

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Left BC, on neoadjuvent

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- The breasts are heterogeneously dense which may obscure small lesions (ACR C)
- Left 2:00 spiculated lesion measuring 2.2x2.2 cm seen 5.2 cm from the nipple, surrounded with multiple well-circumscribed lesions of high density. US showed 2:00 spiculated hypoechoic solid lesion measuring 1.34x1.67 cm, with multiple dilated ducts extending from the nipple toward the lesion, showing internal echogenic contents.
- No right suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- Complementary ultrasound showed no right solid mass lesions.
- Bilateral multiple scattered anechoic simple cysts, with few in the left breast showing internal echogenic contents
- Normal skin thickness and nipple-areola complex.
- Bilateral non-specific axillary LNs.

Opinion:

- Left UOQ subtle progress response of UOQ lesion (BIRADS6)
- Left Retroareolar dilated ducts with ?? inspissated secretions (BIRADS3)
- Bilateral fibrocystic changes (BIRADS 2)

Yours truly

Signature Dr. Yasmin Nada

24/12/2022 11:03 AM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient Name	: الأستاذه / ام الهنا عبدالمعطى بيومى عبدالمحسن	Request No.	: 13003368
Patient No.	: 27404010105141	Request Date	: 11/01/2023
Path No.	: 89/2023	Verification Date	: 24/01/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 48 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data:Left breast cancer, post chemotherapy.

Nature of the specimen: Left simple mastectomy and left SLNs biopsy specimens.

Gross Examination:
- Procedure: Left simple mastectomy specimen; measured 24x16x7 cm, covering skin measured 17x13 cm, nipple and areola were unremarkable grossly.
- Localization: LIQ
- Tumor extent
  - o Number: Ill defined rubbery graiysh white TBS with central firm area
  - o Size: 5x3x2 cm.
  - o Skin involvement: 1.5 cm from skin
- Distance from margins: 2 cm away from deep margin
- Lymph nodes number and size: Three fibrofatty tissue pieces ranging in size from 3x2 cm up to 4.5x2.5 cm, entangling 4 lymph nodes up to 1x1 cm.
- Additional pathologic findings:N/A
- Immunohistochemistry technique: N/A
- Frozen section: Four lymph nodes with therapy effect for paraffin examination.

Microscopic examination:
- Histological type:Invasive tubular carcinoma, cellularity 60%
- Grade (differentiation+ nuclear pleomorphism + mitosis): I (2+2+1)
- Lymph node involvement: Positive metastatic deposits in 1/4 SLNs with capsular invasion
- Resection Margins: Free
- Histologic response to therapy: Pathologic partial response
- Additional pathologic findings: Free nipple and areola

Diagnosis:

Verified by :
walaa ghoniem

غرفة رقم (6) - الدور الأول - مبنى رقم (87) - الممر الأول - شارع القلعة - القاهرة - مصر
Room No. (6) - First Floor Building No. 87 - 5th Corridor - Mar El Qalaa - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / ام الهنا عبدالمعطى بيومى عبدالمحسن	Request No.	: 13003368
Patient No.	: 27404010105141	Request Date	: 11/01/2023
Path No.	: 89/2023	Verification Date	: 24/01/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 48 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Left breast cancer, post chemotherapy; Left simple mastectomy and SLNs biopsy specimens:

- Invasive tubular carcinoma, grade I
- Free nipple and areola
- Free surgical margins
- Positive metastatic deposits in 1/4 SLNs with capsular invasion
- Pathologic partial response
- Stage= ypT2 ypN1

Prof.Dr. Eman Gouda
Dr. Heba Mostafa

Verified by :
walaa ghoniem

Room No. (6) - First Floor Building No. 487 - Nile Corniche - Misr El Qadima - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / ام الهنا عبدالمعطى بيومى عبدالمحسن	Request No.	: 13007700
Patient No.	: 27404010105141	Request Date	: 16/04/2022
Path No.	: 1018/2022	Verification Date	: 26/04/2022
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 48 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical data:
Left breast lesion

Nature of Specimen :
Tru cut biopsy

Gross:
Multiple grayish white tissue cores, longest measured up to 1 cm long. Totally submitted.
Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic:
histopathological examination of sections prepared from the specimen received reveals cores of an infiltrating malignant epithelial growth. The tumour is made up of groups and small sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate [grade II].
Biomarkers.
ER: Strong nuclear reaction in 70% of tumor cells (score 8/8)
PR: Strong nuclear reaction in 70% of tumor cells (score 8/8)
Her-2: Weak incomplete membrane reaction of tumor cells (score 1+)
Ki 67: 10 % nuclear reaction

Diagnosis:
Left breast, Tru cut biopsy;
- Invasive duct carcinoma, GII.
ER : Positive (8/8)

Verified by :
Dr.Marwa mahmoud

Immunohistochemistry
[[DOCTYPE:IHC]]

وزارة الصحة
أمانة المراكز الطبية المتخصصة
مستشفى معهد ناصر
Nasser Institute Hospital

Oncology Center
مركز الأورام
NASSER INSTITUTE
ONCOLOGY CENTER
خطابات أورام

NUCLEAR MEDICINE UNIT
وحدة الطب النووي

Patient Name: Om Elhana AbdElmoaty Bayomi. **Date: 17/8/2022**

^{99m}Tc-MDP BONE SCAN

Desired information : skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- No definite hot spot that account for osseous deposits could be detected allover the surveyed skeleton.
- Degenerative changes are noted in the dorso-lumbar vertebrae.
- Both kidneys are seen

Conclusion:

- ***No definite evidence of metastatic osseous deposits.***

Thanks for reference
Nuclear Medicine Unit

DR/Sherine Mahdy, MD

١٣٥١ كورنيش النيل - أغاخان - شبرا - القاهرة - ت : ٢٢٠٣٩١٦٤ - ٢٢٠٣٩٣٥٦ فاكس : ٢٢٠٣٩٢٣٦
1351Korniesh El Nile - Aghakhan - Shoubra - Cairo Tel : 22039164 - 22039356 Fax : 22039 236

Bone Scan
[[DOCTYPE:BONE_SCAN]]